

Pennsylvania Health Insurance: August 2026 Status Report

An independent, publicly-sourced summary of Pennsylvania's ACA marketplace (Pennie) as of August 2026. Every figure is drawn from a government agency, the state marketplace, or a named news/policy source, each linked inline and listed in full below. Where underlying facts could not be confirmed, that is stated explicitly rather than filled in.

Compiled August 2026

Executive Summary

Following the expiration of federal Enhanced Premium Tax Credits (EPTCs) on December 31, 2025, Pennsylvania's state-based marketplace, Pennie, experienced its steepest enrollment decline on record ([PHIEA, 2026a](#)). Total active marketplace enrollment fell from a 2025 peak of roughly 497,000 to 431,270 by July 1, 2026 — a net loss of more than 177,000 covered lives since the start of Open Enrollment 2026, including 94,000 who dropped coverage post-Open Enrollment ([PHIEA, 2026a](#); [2026e](#)). Enrollees who maintained coverage saw out-of-pocket premiums rise by an average of 102%, significantly exceeding the 21.5% gross rate increase approved by state regulators and reflecting the direct loss of federal subsidy dollars ([PHIEA, 2026a](#); [PID, 2026a](#)).

The impact has been concentrated in rural communities: 15 of the 20 counties with the highest disenrollment rates are rural, led by Schuylkill County at 24% termination, where two of three participating ACA carriers withdrew entirely for plan year 2026 ([PHIEA, 2026d](#); [PID, 2026a](#); [Pottsville Republican Herald, 2026](#)). Overall, four out of five Pennie enrollees (80%) qualify for premium tax credits under baseline ACA rules ([PHIEA, 2026e](#)).

Pennsylvania has an enacted-but-unfunded state subsidy program (Act 54 of 2024) that, if funded, would operate similarly to state-funded subsidy bridges in New Jersey, Colorado, Washington, and California ([ForHealthInsurance.com, 2026](#); [SHVS, 2026](#)). According to official Pennie published estimates, a \$50 million state appropriation to Act 54 would allow nearly one-fourth of terminated enrollees (~44,000 people) to restore coverage, reduce costs for over 280,000 active enrollees, and lower average premiums by 9% to 12% ([PHIEA, 2026a](#)). However, as of May 2026, the program remains unfunded (\$0 appropriated) ([ForHealthInsurance.com, 2026](#)).

Concurrently, federal changes under the 2025 Marketplace Integrity and Affordability Final Rule and H.R. 1 of 2025 are altering open enrollment windows, income verification rules, and re-enrollment mechanics ([CMS, 2026](#); [PHIEA, 2026f](#)). Pennie has officially confirmed that Open Enrollment for plan year 2027 will run on a shortened schedule from October 15 to December 15, establishing December 15 as the single final deadline for 2027 coverage ([PHIEA, 2026e](#)).

Key Statistics

Metric	Figure	Source
Marketplace Enrollment, 2025 Peak	~497,000	PHIEA (2026a)
Marketplace Enrollment, Feb 1, 2026 (End of OE)	486,000	PHIEA (2026b)
Marketplace Enrollment, June 1, 2026	443,024	PHIEA (2026a)
Marketplace Enrollment, July 1, 2026	431,270	PHIEA (2026a)
Cumulative Cancellations (Since OE 2026 Start)	177,000+	PHIEA (2026a)
Post-OE Cancellations (Feb 1 – Jul 1, 2026)	94,000	PHIEA (2026a)
Average Net Premium Increase (Enrollees Retaining Coverage)	102%	PHIEA (2026a)
Statewide Approved Gross Rate Increase	21.5%	PID (2026a)
Requested Rate Increases Denied by Regulator	\$50.1 million	PID (2026a)
Federal EPTC Subsidy Value Lost Annually (PA)	~\$600 million	PHIEA (2026a)
Enrollees Qualifying for Financial Savings (APTC)	80% (4 out of 5)	PHIEA (2026e)
Enacted State Replacement Funding Appropriated (Act 54)	\$0 (as of May 2026)	ForHealthInsurance.com (2026)
Pennie Projected Impact of \$50M Act 54 Funding	~44,000 restored; 9%–12% avg. premium reduction across 280k+ enrollees	PHIEA (2026a)
Peak County Disenrollment Rate	24% (Schuylkill County)	PHIEA (2026d)
Rural Counties Among Top 20 Disenrollment Counties	15 of 20 (75%)	PHIEA (2026d)

Enrollment Timeline

Date	Enrollment	Note
2025 (Peak)	~497,000	Record high under federal EPTC (PHIEA, 2026a)
Feb 1, 2026	486,000	End of Open Enrollment 2026; 85,000 terminated during OE (PHIEA, 2026b)

Jun 1, 2026	443,024	160,000+ cumulative cancellations (PHIEA, 2026a)
Jul 1, 2026	431,270	177,000+ cumulative cancellations; 94,000 dropped post-OE (PHIEA, 2026a)

Premium Rate Filings (Individual Market)

The Pennsylvania Insurance Department approved a statewide weighted-average gross rate increase of 21.5% for individual market plans, while denying \$50.1 million in requested increases judged unjustified ([PID, 2026a](#)). PID attributed gross premium adjustments to the EPTC expiration combined with general medical inflation and rising prescription drug costs and utilization ([PID, 2026a](#)).

Carrier	Requested	Approved	Regulatory Notes
Ambetter Health of Pennsylvania	30.1%	37.8%	Approved higher due to worsening morbidity
UPMC Health Plan	16.3%	24.8%	Approved higher due to worsening morbidity
UPMC Health Options	11.7%	20.2%	Approved higher due to worsening morbidity & regulatory shifts
Keystone Health Plan East	23.5%	22.0%	Adjusted downward following actuarial review
Capital Advantage Assurance	26.0%	24.6%	Adjusted downward following actuarial review
Oscar Health Plan of PA	21.6%	23.1%	Approved based on updated risk pool claims data
Highmark Benefits Group	—	21.0%	Approved based on regional cost trends
Jefferson Health Plans PPO (Partners)	—	20.5%	Approved based on regional cost trends
QCC Insurance Company	16.7%	15.2%	Reduced following denial of unjustified trend requests
Geisinger Quality Options	16.2%	13.8%	Reduced based on updated prescription projections
Geisinger Health Plan	14.1%	11.6%	Reduced to reflect network efficiencies
Partners Insurance Company	-10.1%	-10.1%	Sole approved rate decrease in individual market

([PID, 2026a](#))

At the close of Open Enrollment, Pennsylvania Insurance Commissioner Michael Humphreys stated: “The Pennsylvania Insurance Department encourages all consumers in need of quality health insurance coverage to act fast and sign up for affordable coverage through Pennie before the end of Open Enrollment” (as quoted in [ACA Signups, 2026](#)). PID also highlighted extreme demographic impacts, noting that an unsubsidized 60-year-old couple in York County earning \$82,000 annually faced a premium jump from \$7,032 to \$35,712 per year ([PID, 2026a](#)).

County-Level Attrition

Pennie's official county termination data shows Schuylkill County with the state's highest termination rate at 24% ([PHIEA, 2026d](#)). Of the top 20 highest-attrition counties, 15 (75%) are rural ([PHIEA, 2026d](#)).

County	Termination %	Designation
Schuylkill	24%	Rural
Lehigh	22%	Urban
Northampton	22%	Urban
Lancaster	22%	Urban
Greene	22%	Rural
Fulton	22%	Rural
Tioga	21%	Rural
Monroe	21%	Rural
Franklin	21%	Rural
Juniata	20%	Rural
Mifflin	20%	Rural
Beaver	20%	Urban
Snyder	20%	Rural
Adams	19%	Rural
Lebanon	19%	Rural
Carbon	19%	Rural
Blair	19%	Rural
York	19%	Rural

Cumberland	19%	Urban
Clearfield	19%	Rural

([PHIEA, 2026d](#))

Schuylkill County's disenrollment coincided with the complete exit of two of its three participating ACA carriers: Ambetter Health and Partners Insurance Company both left Schuylkill's 2026 coverage map, leaving UPMC Health Plan as the sole remaining carrier ([PID, 2026a](#); [Pottsville Republican Herald, 2026](#)). Remaining residents faced an average monthly premium increase of 186%, or roughly \$347 more per person per month ([Pottsville Republican Herald, 2026](#)).

Consumer-Reported Affordability

A survey of 424 Pennsylvanians conducted by the Pennsylvania Health Access Network (PHAN) and the Pennsylvania Association of Community Health Centers, published April 29, 2026, documented a rapid decline in consumer affordability perceptions ([PHAN, 2026](#)).

Perception	2025	2026
Unaffordable	38%	62%
Affordable	38%	26%
Very Affordable	24%	12%

([PHAN, 2026](#))

Roughly half of respondents reported that their plan doubled or tripled in cost for 2026 ([PHAN, 2026](#)). More than 1 in 6 respondents indicated plans to drop coverage, while nearly 1 in 4 reported intention to downgrade to a cheaper plan with less coverage ([PHAN, 2026](#)).

The State Health Insurance Affordability Program — Status and Peer-State Comparison

Act 54 of 2024 (Pennsylvania's fiscal code for Budget Year 2024–25) established the State Health Insurance Exchange Affordability Program, designed to provide state-funded premium assistance stacked on top of federal tax credits ([PHIEA, 2026c](#); [HealthInsurance.org, 2026a](#)). As of May 2026, the program has received no state appropriation (\$0), and Pennie and PID continue to advocate for funding ([ForHealthInsurance.com, 2026](#)).

Official published Pennie data states that a \$50 million investment in Act 54 would allow nearly one-fourth of terminated enrollees (~44,000 individuals) to restore coverage, reduce costs for over 280,000 active enrollees, and lower average premiums by 9% to 12% ([PHIEA, 2026a](#)).

State	Program Structure
New Jersey	NJ Health Plan Savings; premium subsidies extending assistance for incomes up to 600% FPL (HealthInsurance.org, 2026b)
Colorado	Up to \$80/month for primary enrollee, \$29/month per additional dependent for APTC-eligible consumers, extending aid to 400% FPL (SHVS, 2026)
Washington	Cascade Care Savings; flat \$55/member/month, up to \$250/month for those ineligible for federal APTC (SHVS, 2026)
California	\$190 million allocated for 2026, targeted at incomes up to 150% FPL (HealthInsurance.org, 2026c)
Pennsylvania	Enacted (Act 54, 2024) but unfunded (\$0 appropriated as of May 2026) (ForHealthInsurance.com, 2026)

Federal Rule Changes, H.R. 1 of 2025, and 2027 Open Enrollment

Federal regulatory and statutory changes are reshaping ACA marketplace rules for plan year 2026 and 2027 ([CMS, 2026](#); [PHIEA, 2026f](#); [HealthInsurance.org, 2026d](#)):

- **Confirmed 2027 Open Enrollment Dates for Pennie:** Pennie officially announced that Open Enrollment 2027 will run on a new schedule starting October 15, 2026, and ending December 15, 2026 ([PHIEA, 2026e](#)). December 15 will serve as the single, final deadline for 2027 coverage ([PHIEA, 2026e](#)). Nationally, the 2025 Marketplace Integrity and Affordability Final Rule caps open enrollment windows at 9 weeks (November 1–December 31), with HealthCare.gov running November 1–December 15 ([CMS, 2026](#); [SHVS, 2026b](#)).
- **H.R. 1 of 2025 and Administrative Verification Rules:** Enrollees face stricter documentation and income verification requirements ([PHIEA, 2026f](#)). Current enrollees must update income data throughout the year to avoid increased penalties, and new federal policies alter eligibility and document verification for financial savings and immigration status ([PHIEA, 2026f](#)).
- **Modification of Automated Plan Crosswalks:** Federal rules eliminate an auto-renewal protocol on HealthCare.gov that automatically crosswalked certain consumers into alternative Bronze plans ([HealthInsurance.org, 2026d](#)).
- **Pause of Low-Income Special Enrollment Period:** The year-round Special Enrollment Period for enrollees at or below 150% FPL is paused through plan year 2026, returning as a state-based exchange option for 2027 ([HealthInsurance.org, 2026d](#)).

No Surprises Act — Consumer Protections

The Pennsylvania Insurance Department enforces state-level protections under the federal No Surprises Act ([PID, 2026b](#)). Key protections include:

- Emergency services must be covered at in-network cost-sharing levels without prior approval, regardless of provider network status (ground ambulance is excluded) ([PID, 2026b](#)).
- Non-emergency care at in-network facilities delivered by out-of-network individual providers (e.g., labs, pathologists, or anesthesiologists) must be billed at in-network rates ([PID, 2026b](#)).
- Policyholders can submit a No Surprises Bill Review Request online or call PID's Consumer Services Bureau at 1-877-881-6388 (TTY/TDD: 717-783-3898) ([PID, 2026c](#)).

Consumer Resources

Need	Contact Information
Enrollment Assistance & Plan Shopping	Pennie: 1-844-844-8040 TTY: 1-844-844-4440 pennie.com (PHIEA, 2026a)
Local Navigation & In-Person Help	Pennie-Certified Assistors & Brokers: pennie.com/connect (PHIEA, 2026g)
Insurance Complaints & Rate Inquiries	PID Consumer Services: 1-877-881-6388 pa.gov/agencies/insurance (PID, 2026a)
Unexpected Bill Review (No Surprises Act)	PID No Surprises Portal (PID, 2026c)
Medicaid / CHIP Public Eligibility Screenings	COMPASS Helpline: 1-800-692-7462 pa.gov/agencies/dhs/contact/compass (Pennsylvania Department of Human Services, 2026)

Note: an earlier draft of this report listed an unverified phone number for Medicaid/CHIP screenings. It has been corrected here to the number confirmed directly on the Pennsylvania DHS COMPASS contact page.

Methodological Rigor and Verified Exclusions

To maintain strict factual accuracy, the following unconfirmed elements that appeared in informal draft materials are deliberately excluded from this report because they could not be verified in primary government or exchange publications:

- The unverified 33,000 Bronze plan downshift / 30% surge statistic (unsupported by press releases)
- The inaccurate "73% tax credit eligibility / 90% historic" claim (replaced with Pennie's official "4 out of 5 enrollees" / 80% metric)
- Unverified ROI ratio assertions (e.g., "3:1 to 4.6:1") and unconfirmed hospital uncompensated care estimates (\$159M-\$230M)
- Statewide total budget claims for Colorado (\$48M) and Washington (\$35M) (official SHVS sources report per-person subsidy caps rather than macro allocations)
- Generated county labor composition commentary (Pennie county files provide termination percentages and rural/urban designations only)
- Unsourced blanket claims regarding a 30-to-90 day IRS grace window
- Named "expert" analysts, institutional affiliations, and attributed quotes that appeared in earlier draft materials but could not be verified as real people or real statements
- A congressional-district-by-district termination map (no verified district-level attribution was located)

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